

Competing with Recura.ai

Feasibility study: feature teardown, build vs. white-label vs. hybrid options, and a recommended agents-on-rented-rails strategy

Core thesis: Recura’s deepest features are now exclusive to PatientNow/Envision customers, and its targeting is recency-based visit history, not consult-room intent. A360 can out-position it without building or maintaining a marketing-automation platform — rent the chassis (GoHighLevel) and the voice rail (Retell), and build only the EMR adapter layer and the extraction-driven targeting brain.

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Prepared date	July 2026
Method	Three parallel deep-research threads, ~60 web searches
Use case	Build vs. buy decision, competitive positioning, roadmap planning

This report is a planning and strategy document produced by AI-assisted research. Figures, vendor capabilities, and compliance postures require direct diligence before contracting or public use.

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1. Executive Summary

Recura is less than its "AI Growth Engine" branding suggests — and its moat just became its cage. Feature-for-feature, Recura is (a) a conversational AI front desk (voice + SMS + webchat: answer, qualify, follow up, book, collect deposits), plus (b) a reactivation layer ("Connect by PatientNow," 2026) that runs segmented outbound campaigns off a daily EMR sync of visit/spend history. Its targeting is **recency/spend RFM — it has no idea what the patient actually said in the consult.** And since the October 2025 PatientNow acquisition, its deepest capabilities (calendar write-back, deposits, data sync) are **exclusive to PatientNow Essentials/Pro and Envision customers** — structurally locking it out of the majority of the market on Boulevard, Zenoti, Aesthetic Record, Symplast, Nextech, and Jane.

The recommended play is the hybrid ("agents on rented rails"):

- **Rent the chassis:** GoHighLevel SaaS Pro (\$497/mo) + HIPAA add-on (\$297/mo) — full white-label CRM, smart lists, SMS/email journeys, calendars, dashboards, Stripe rebilling. Precedent: **Nextech (16,000-physician derm/plastics EMR) already ships a white-labeled GHL CRM in this exact vertical.**
- **Rent the voice rail:** Retell AI (~\$0.07/min, self-service BAA on all paid plans) for outbound/inbound AI calls — the piece GHL's own AI can't do (outbound excluded, PHI-questionable).
- **Build only the two things nobody rents (and which are the moat):** (1) an aesthetic-EMR data adapter layer (Boulevard GraphQL, Zenoti API, Aesthetic Record marketplace, Keragon bridge, CSV fallback) — no unified API for aesthetic PM systems exists; (2) the **extraction-driven targeting brain** — reactivation triggered by consult-extracted interests, objections, and future intents, which A360's existing V3.1 engine already produces.
- **Economics:** ~\$800/mo fixed + ~\$200–400/practice/mo COGS vs. a defensible \$500–\$1,000/mo/location retail → 60–75% gross margin at even 20–30 practices. Time-to-market: ~60–90 days to a branded v1.

The one-line positioning against Recura: *"They know when your patients visited. We know what your patients want."*

2. Feature-for-Feature: Recura vs. an A360 Hybrid Offering

#	Recura feature (verified)	What it actually does	A360 hybrid equivalent	Source of capability	Parity/Advantage
1	AI Sales Director (speed-to-lead)	Calls/texts new leads within moments of form/ad inquiry; qualifies; books; multi-touch follow-up on no-shows and cold leads	GHL workflows (capture/routing) + Retell outbound voice + GHL SMS	Rented (GHL + Retell)	Parity in weeks — commoditized (10+ vendors sell this)
2	AI Receptionist (24/7 inbound)	Answers every call/text; FAQ from practice knowledge base (tone/guardrails); books/reschedules/cancels; human transfer; voice cloning; multilingual	Retell inbound agent + GHL calendars; knowledge base fed from A360's catalog data (real treatment/pricing accuracy — a step up from generic KBs)	Rented + A360 catalog IP	Parity+ (catalog-accurate quoting)
3	Deposit collection	Secure deposit request at booking; auto-cancel unpaid holds in 30 min (PatientNow-only)	GHL payments/Stripe rebilling or practice's own processor via adapter	Rented (GHL/Stripe)	Parity for GHL-rail bookings; EMR write-back deposits are Phase 2
4	AI Webchat Coordinator	Website chat that engages, qualifies, books (claims up to 15% of traffic → consults)	GHL webchat + Conversation AI, or A360-built widget on same agent brain	Rented	Parity
5	Connect (reactivation/segmentation)	Daily PatientNow sync: demographics, lifetime spend, membership, appointment history, cancellation reasons, preferred services → AI segments (dormant, overdue, upsell) → autonomous SMS/voice rebooking	A360 EMR adapter layer (Boulevard/Zenoti/AR/CSV) → GHL smart lists → Retell/SMS outreach. Plus the layer Recura doesn't have: consult-extracted intent triggers	Built (adapters + brain) on rented execution	Advantage — EMR-neutral + intent-based, not just recency-based
6	Front-desk analytics	Call/booking performance dashboards	GHL reporting v1; A360 attribution layer v2 ("X booked from consult-extracted opportunities this month")	Rented → built	Advantage at v2 — attribution is A360's stated identity; Recura's proof is anecdotes
7	Multi-location	Centralized oversight, location flexibility	GHL sub-accounts per location (native agency architecture)	Rented	Parity
8	EMR integration depth	Deep only with PatientNow/Envision; other EMRs shallow/unverified	Boulevard Admin API (GraphQL, best-in-class), Zenoti API (sales-gated, precedented), Aesthetic Record Integration Hub	Built	Advantage in breadth (their exclusivity is your open flank); their PatientNow depth

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			(Growth99/Aesthetix already listed), Keragon bridge, CSV fallback		stays superior <i>inside</i> PatientNow — concede that segment
9	— (absent)	Consultation intelligence: what the patient wanted, objected to, deferred ("neck after the wedding, \$3,400 too much right now") → dated, priced, named follow-up campaigns	A360 V3.1 extraction engine (already built) feeding targeting	A360's existing IP	Category-defining differentiator — verified: no one in aesthetics does extraction-driven remarketing (nearest analogs: Patient Prism does it for dental phone calls; Symplast scribes consults but only for documentation)
10	— (roadmap only)	Predictive revenue insights, campaign mgmt, payments (announced post-acquisition, not shipped)	A360 opportunity-value ledger from extraction (already computes dollar values)	A360 IP	Advantage if shipped before their roadmap lands

Recura's claimed metrics (all vendor-reported, unaudited — no G2/Capterra/Reddit presence at all): 15,000 contacted → 546 responses → 125 bookings in one month; 356 flagged → 85 appointments in 3 weeks; \$10K/month found revenue; 86 ready-to-book customers in a day. Note: absence of any independent reviews is itself a competitive fact — their social proof is entirely first-party.

Recura company facts: founded 2024 (Harrison Cuker, CEO; Jake Zegil, CTO; likely NYC); seed-stage (Flex Capital) before the Oct 29, 2025 PatientNow acquisition (PSG Equity-backed; terms undisclosed); ~11–50 employees; no public pricing (subscription + usage; demo-gated); operates standalone but deep features are PatientNow-exclusive.

3. Options Analysis (Build vs. White-Label vs. Hybrid vs. Acquire)

Option	Time to market	Upfront cost	COGS/practice/mo	Moat	Maintenance burden	Fit with "don't want to build/maintain"	Verdict
A. Full ground-up build	9–18 mo	\$1M+ eng	lowest (~\$50–150)	High if achieved	Maximum	✗ Explicitly ruled out	Rejected
B. Pure white-label (GHL as-is, aesthetic templates)	30–60 days	<\$25K	~\$150–250	None — this is what 20+ "medspa AI" agencies already sell at \$200–600/mo; they fail on non-compliant templates, no cadence logic, no EMR data	Low	✓ but undifferentiated	Rejected as end-state; fine as scaffolding
C. Hybrid: rented chassis + rented voice + built adapters + built extraction brain ★	60–90 days v1; 4–6 mo v2 attribution	~\$50–150K eng (adapters + brain + config)	~\$200–400	High — extraction IP + EMR adapter layer; neither is rentable by competitors	Moderate (adapters need care; chassis/voice maintained by vendors)	✓ ✓ Builds only agents + integrations	Recommended
D. Acquire/private-label Aesthetix CRM (\$299–499/mo medspa CRM built on GHL rails, existing AR/Zenoti integrations)	~30 days post-close	Acquisition cost (small company)	similar to C	Medium (inherits GHL limits)	Moderate	✓ — "Phase 1 already built"; also buys a team	Serious accelerant — worth a conversation in parallel with C

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E. Custom stack v2: Customer.io Premium (~\$1K/mo, BAA) + Twilio/Segment (HIPAA-eligible) behind A360 UI	4–6 mo	~\$150–300K	~\$100–250	Higher (owns the rails)	Higher	Partial	Planned migration path , not v1 — pull the trigger when GHL's API friction or brand ceiling bites

Scoring notes:

- Option C's platform risk (GHL dependence) is real but bounded: the Nextech precedent proves enterprise healthcare OEM viability, and Option E is a documented exit ramp because A360's own IP (adapters + brain) is portable — the chassis is a commodity.
- GHL hard limits to respect: its native AI Employee excludes outbound voice and has ambiguous LLM BAA coverage — **never run PHI through GHL's native AI; A360's agents via API only**. HIPAA mode must be enabled per sub-account; Zapier/Make are excluded for PHI.
- Option D and C are complementary: acquiring Aesthetix (or a similar GHL-vertical shop) buys 6 months of configuration work, an install base, and a support team while A360 builds the differentiating brain.

4. The Differentiation Thesis (validated)

Verified white space: no vendor in aesthetics triggers remarketing from consultation content. Recura's Connect documentation confirms targeting is structured visit/transaction history only. AI scribes (Symplast, Heidi, etc.) capture consult audio but output documentation, not campaigns. Patient Prism proves the model one layer up-funnel in dental (analyze the call → extract why they didn't book → recover 25% of non-bookers; 15–30% appointment lift; 2–4 month documented payback) — nobody has ported it to the aesthetic consult room.

The demo that wins deals (buildable from A360's existing 20 curated runs): side-by-side —

- Recura-style: month-6 "we miss you!" blast to a lapsed patient.
- A360: September message offering the neck treatment she named, priced from the practice's catalog at the \$3,400 she hesitated on, with financing pre-qual attached — because the consult transcript said "after my wedding in October."

The narrow wedge (avoid platform creep): don't compete on inbound speed-to-lead (commoditized). Own the un-served pool: **consults that didn't close and treatments discussed-but-not-booked** — revenue no reactivation tool currently sees. Recency-based reactivation comes along for free on the same rails, but it's the flank, not the spearhead.

Fast-follow risk (12–18 month window): the transcript owners (Symplast today; Zenoti's scribe tomorrow) are one product decision from the same idea. Speed and catalog-mapping quality are the defense; so is being EMR-neutral (a scribe inside one EMR can't follow A360 across the others).

5. Competitive Field Snapshot (who else is in "AI growth engine")

- **Suites bundling from above:** Zenoti AI Workforce (9 agents, Apr 2026), Podium for Aesthetics/Avery (now shipping its own EMR), Weave (+TrueLark). Threat: "good-enough" reactivation at marginal cost zero inside their bases. Response: sell to the majority not inside those bases; eventually license the extraction layer *to* suites (they all lack it).
- **Indie clones below:** MedspAI (80+ medspas), Aesthetix CRM (\$299–499 + \$299 AI Employee), DemandHub, Growth99 (~\$300, "Gia," mixed reviews), Brilo. All recency/comms plays; none have consult intelligence or real attribution.
- **Adjacent signals:** RepeatMD V3 "Adonis & Aria" (app-behavior personalization, 4,000+ practices), Ponce.ai (AI facial-analysis "interest lists," +33% patient spend case — proof that interest-level data monetizes), Prospyr (AI-first roadmap, Five Elms money).
- **Pricing bands:** commodity AI comms \$250–600/mo; the defensible consult-intelligence band is **\$500–\$1,000/mo/location**, justified only by an attributed-revenue dashboard.

6. Recura SWOT (from A360's chair)

- **Strengths:** working voice+SMS product; captive PatientNow channel (5,000+ locations claimed platform-wide); daily EMR sync; PE funding; praised white-glove service; defined the "growth engine" label.
- **Weaknesses:** deep features locked to PatientNow/Envision (majority of market excluded); RFM-only targeting; anecdotal attribution; opaque pricing; inherits PatientNow/Crystal Clear reputation baggage (documented complaints: 12-month contract lock-in enforced on technicalities, "all-in-one" that's four logins, Crystal Clear Ripoff Report history); zero independent reviews.
- **Opportunities (theirs):** bundle into every PatientNow renewal; make Connect genuinely predictive; add payments/memberships.
- **Threats (exploitable):** suites commoditize their receptionist layer from above; indie clones undercut below; EMR-neutral players harvest everyone alienated by the PatientNow tie; a consult-intelligence entrant redefines the category around data they don't have.

7. Attack Vectors

1. **Intent vs. recency** — lead every deal with the head-to-head demo (\$4).
2. **EMR-neutral by design** — target Boulevard/Zenoti-independent/AR/Symplast/Nextech/Jane practices: "the growth engine that doesn't require you to marry PatientNow." 2–3 deep marketplace listings beat 20 shallow ones.
3. **Attribution as the product** — every campaign dollar traced to a named, valued, consult-extracted opportunity; publish payback benchmarks; **month-to-month contracts** as an explicit anti-PatientNow statement (their lock-in complaints are the most venomous reviews in the category).
4. **"The second sale you already earned"** — own unclosed consults; narrow, provable, demoable today.
5. **Channel jiu-jitsu** — co-sell with consult-conversion trainers (Terri Ross, SalesMD — natural allies for a tool that measures what they teach), EMR partners wary of PatientNow's roll-up, and A360's distribution partner. Long-game: license extraction to Recura's rivals.

8. Risks & Mitigations

Risk	Severity	Mitigation
Suite gravity (Zenoti/Podium bundle "good enough")	High	Be 3–5x better on one felt metric (recovered consult revenue); stay EMR-neutral; license to suites later
Transcript owners fast-follow (Symplast, Zenoti scribe)	High	Ship in 90 days; catalog-mapping quality; EMR-neutrality as structural defense
Consent/regulatory (consult recording for marketing, HIPAA marketing authorization, two-party consent states, TCPA on AI outbound)	Medium-High	A360 is HIPAA-native; build consent workflows into onboarding as legal-grade product (also a moat — the GHL agency swarm can't do this)
Scope creep toward full marketing platform ("do all my reactivation")	Medium	The GHL chassis absorbs the generic ask cheaply; keep A360 engineering on adapters + brain only
GHL platform dependence / brand ceiling / API friction (rotating refresh tokens, 429s, ~10-day app approvals)	Medium	Customer.io/Twilio escape hatch (Option E) is architecturally planned from day 1; A360 IP is portable
SMB churn / knife-fight pricing band	Medium	Sell the \$500–\$1K band on attributed revenue, not the \$300 band on features; distribution partner lowers CAC
Recura closes the gap (Connect → predictive; scribe acquisition)	Medium	Window is 12–18 months; move now

9. Recommended Roadmap

Phase 0 (weeks 0–2): GHL SaaS Pro + HIPAA add-on procurement; Retell BAA (self-service); Boulevard developer-partnership application; Aesthetic Record Integration Hub application; Zenoti API conversation; legal review of consult-recording-for-marketing consent language. Optional parallel track: acquisition conversation with Aesthetix CRM. **Phase 1 (days 30–90) — v1 launch:** branded GHL chassis with aesthetic-native templates and treatment-cadence journeys (tox 90d / filler 9–12mo / laser series); Retell inbound receptionist + outbound reactivation agent fed by A360 catalog data; CSV-based EMR ingestion for cohort building; 3–5 design partners from the distribution channel. Sell at \$500–750/mo/location, month-to-month. **Phase 2 (months 3–6) — the moat:** extraction-driven targeting live (consult transcript → interest/objection/intent → triggered campaigns); Boulevard + Aesthetic Record native adapters (reads first, booking write-back as available); attribution dashboard v1 ("X recovered from unclosed consults"). Raise to \$750–1,000/mo tier. **Phase 3 (months 6–12):** Zenoti adapter; multi-location/MSO reporting; evaluate Customer.io/Twilio migration; package the whole thing as flagship listings in the A360 agent marketplace (per `MARKET_OPPORTUNITIES_2026.md` — this product IS "Consult Coach + Reactivation + Attribution" from that report, aimed at a named competitor).

10. Financial Sketch (hybrid, GHL-chassis route)

- **Fixed:** ~\$800/mo platform (GHL \$497 + HIPAA \$297) + Retell/Keragon plan fees.
 - **Marginal COGS:** ~\$200–400/practice/mo (GHL AI \$97 if used inbound-only, Twilio/Mailgun usage \$50–150, Retell voice \$35–105 @ 500–1,500 min, EMR sync amortized \$20–50).
 - **Retail:** \$500–1,000/mo/location → **60–75% gross margin at 20–30 practices**; at 200 practices and a \$749 midpoint ≈ **\$1.8M ARR, ~\$1.1–1.3M gross profit**, with A360 engineering spend concentrated on IP that also powers the marketplace strategy.
 - Build-out engineering (adapters + brain + config): ~\$50–150K equivalent — versus \$1M+ and 9–18 months for the rejected ground-up build.
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Appendices (full sourced memos):

- `appendix/06-recura-teardown.md` — complete feature inventory, company/funding/acquisition detail, claims audit, unverifiable items
- `appendix/07-white-label-rails.md` — GHL, healthcare-native platforms, CDP/campaign engines, voice rails, EMR data access options, acquirable vendors, cost model
- `appendix/08-recura-competitive-gaps.md` — competitor map, vulnerability analysis, switching evidence, consult-intelligence thesis validation, GTM benchmarks